

Patient 17**Report A**

6552998 04/07/2016 MR Head

6552998 04/07/2016 MR MRA

Indication: Right thalamichaemorrhage - MRI to assess microbleeds

Findings: Comparison with previous CT imaging of 30/06/2016.

Right striato-capsular/ thalamic microhaemorrhage again noted, with some localised mass effect effacing the right lateral ventricle, third ventricle and lateral aspect of the right midbrain. No hydrocephalus. There are multiple foci of low signal seen on gradient echo imaging, mainly in a central location around the basal ganglia, but with some scattered peripheral, Cerebellar and pontine foci also noted. These findings are Consistent with hypertensive microhaemorrhages, although an element of amyloid angiopathy is also a possibility. There is periventricular and deep white matter low attenuation consistent with moderate chronic small vessel disease. No new acute parenchymal haemorrhage is identified. The circle of Willis, proximal PCA, MCA and ACA are patent with no evidence of intracranial aneurysm, stenosis or occlusion.

Report B

654970530/06/2016 CT Acute Stroke

For Thrombolysis? Potentially

Report:

Time 1st Called: 1550

Time plain CT available on PACS: 1555

Time of verbal report to stroke registrar:1555

Indication: Left-sided hemiparesis

CT HEAD Haemorrhage: Yes. There is an acute intraparenchymal haematoma involving the right thalamus and striato-capsular region measuring approximately 2,5 cm x 1,9 cm.

Intravascular thrombus: No

Acute Infarct: No acute large vessel territorial infarct is identified. Other findings and comments: Scattered white matter hypoattenuating lesions represent probable microangiopathic ischaemic changes.

CT ANGIOGRAM

Window settings: (intraluminal HU=340; W=680; L=255)

CA Stenoses NASCET method, measurements on axial 1mm sections) Aortic arch and Subclavian arteries: Normal

R CCA/CA: Normal

LCCA/ICA: Normal

RVA: Normal

LVA: Normal. The left vertebral artery originates from the aortic arch immediately proximal to the left subclavian artery.
Intracranial Arteries: Normal. No aneurysms or vascular malformations are identified on this nonphased study. There is no angiographic evidence of active bleeding.

No Occlusions or stenoses are shown. Other findings and comments: Several subcentimetre cervical lymph nodes are noted, likely reactive.

Conclusion:

Findings consistent with a right striatocapsular thalamic bleed.

This is likely hypertensive in aetiology.

No evidence of active bleeding.

Unremarkable cerebral and carotid angiogram.